



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MEDICINE SUBJECT WISE TEST , DATED ON 11th JUNE 2026

QUESTIONS

- Q1. A 26-year-old woman presents with 6 months of fluctuating drooping of both eyelids and intermittent double vision. Symptoms worsen by evening and improve after rest. Neurological examination reveals fatigable ptosis. Deep tendon reflexes are normal, and sensory examination is unremarkable. The most likely pathogenic antibody is:
- a) Anti-voltage-gated calcium channel antibody
 - b) Anti-acetylcholine receptor antibody
 - c) Anti-GM1 antibody
 - d) Anti-aquaporin-4 antibody
- Q2. A 32-year-old man with known myasthenia gravis develops fever and productive cough. Over the next 24 hours he develops worsening dyspnea and difficulty counting beyond 6 in a single breath. Oxygen saturation is 91% on room air. The most important immediate investigation is:
- a) CT chest
 - b) Forced vital capacity measurement
 - c) Repetitive nerve stimulation
 - d) Anti-AChR antibody titer
- Q3. A 24-year-old man develops progressive weakness in both legs 10 days after an episode of bloody diarrhea. Over the next 3 days, weakness ascends to involve the upper limbs. Examination reveals generalized areflexia. Which organism most likely triggered his illness?
- a) *Salmonella typhi*
 - b) *Campylobacter jejuni*
 - c) *Vibrio cholerae*
 - d) *Staphylococcus aureus*
- Q4. A 35-year-old woman presents with acute ascending paralysis. CSF analysis performed one week after symptom onset shows protein 180 mg/dL and 2 lymphocytes/mm³. This CSF finding is best described as:
- a) Neutrophilic pleocytosis



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- b) Albuminocytologic dissociation
- c) Oligoclonal band positivity
- d) Xanthochromia

Q5. A 67-year-old hypertensive man develops sudden onset right-sided weakness and inability to speak while having breakfast. He reaches the emergency department within 90 minutes. Non-contrast CT brain shows no hemorrhage. The next best management is:

- a) Aspirin 325 mg immediately
- b) IV thrombolysis after eligibility assessment
- c) Oral anticoagulation
- d) Decompressive craniectomy

Q6. A 58-year-old man presents with vertigo, ipsilateral facial numbness, contralateral loss of pain sensation in the body, dysphagia, and Horner syndrome. The most likely vascular lesion involves:

- a) Anterior cerebral artery
- b) Middle cerebral artery
- c) Posterior inferior cerebellar artery
- d) Lenticulostriate artery

Q7. A 72-year-old man presents with exertional chest pain and syncope. Examination reveals a harsh ejection systolic murmur in the right second intercostal space radiating to the carotids. Carotid pulse is slow-rising and of low amplitude. The most likely diagnosis is:

- a) Mitral regurgitation
- b) Aortic stenosis
- c) Hypertrophic cardiomyopathy
- d) Ventricular septal defect

Q8. A 58-year-old man with type 2 diabetes has persistent albuminuria and eGFR of 42



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mL/min/1.73m². Blood pressure is 148/92 mmHg. Which medication has the strongest evidence for slowing progression of kidney disease?

- a) Digoxin
- b) Ramipril
- c) Hydrochlorothiazide
- d) Amlodipine

Q9. A 22-year-old woman experiences wheezing and chest tightness twice weekly. Spirometry is normal between episodes. She has no nocturnal symptoms. According to current GINA recommendations, the preferred reliever is:

- a) Salbutamol alone
- b) As-needed low-dose ICS-formoterol
- c) Oral prednisolone
- d) Theophylline

Q10. A 42-year-old woman reports pain and swelling involving both wrists, MCP, and PIP joints for 6 months. Morning stiffness lasts 2 hours. ESR is elevated. Which antibody is most specific for her disease?

- a) Rheumatoid factor
- b) ANA
- c) Anti-CCP antibody
- d) Anti-dsDNA antibody

Q11. A 24-year-old school teacher complains of intermittent diplopia and drooping of both eyelids for the last 8 months. Symptoms are minimal in the morning but worsen by evening. Examination reveals fatigable ptosis. Deep tendon reflexes and sensory examination are normal. Which investigation is most likely to confirm the diagnosis?

- a) Nerve conduction study showing conduction block
- b) Anti-acetylcholine receptor antibody assay
- c) CSF examination



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d) MRI brain with contrast

Q12. A 32-year-old woman with generalized myasthenia gravis develops fever and productive cough. Over the next 24 hours, she develops increasing breathlessness and difficulty speaking full sentences. Oxygen saturation is 93% on room air. Which parameter is most useful in deciding the need for ventilatory support?

- a) ESR
- b) Forced vital capacity
- c) Serum CK
- d) Anti-AChR antibody titer

Q13. A 29-year-old man presents with ptosis, dysphagia, and proximal muscle weakness. CT chest reveals an anterior mediastinal mass. The most likely associated lesion is:

- a) Teratoma
- b) Thymoma
- c) Lymphoma
- d) Neuroblastoma

Q14. A patient with stable myasthenia gravis develops severe preeclampsia and receives intravenous magnesium sulfate. Within hours, she develops worsening weakness and respiratory distress. Magnesium worsens MG primarily by:

- a) Blocking acetylcholine release at the neuromuscular junction
- b) Increasing acetylcholine breakdown
- c) Causing muscle necrosis
- d) Blocking dopamine receptors

Q15. A 30-year-old man develops progressive ascending weakness 2 weeks after an episode of diarrhea. Examination reveals bilateral lower motor neuron facial palsy and generalized areflexia. The most likely diagnosis is:

- a) Chronic inflammatory demyelinating polyneuropathy



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- b) Guillain-Barré syndrome
- c) Myasthenia gravis
- d) Botulism

Q16. A 35-year-old woman presents with acute ascending paralysis. CSF examination performed 7 days after symptom onset reveals protein 180 mg/dL and 2 lymphocytes/mm³. This finding is termed:

- a) Pleocytosis
- b) Albuminocytologic dissociation
- c) Froin syndrome
- d) Oligoclonal band positivity

Q17. A patient with GBS suddenly develops severe fluctuations in blood pressure and episodic bradycardia requiring ICU admission. The most likely explanation is:

- a) Raised intracranial pressure
- b) Brainstem infarction
- c) Autonomic dysfunction
- d) SIADH

Q18. A 40-year-old man with severe GBS is unable to walk and has rapidly progressive weakness. Which treatment has been shown to shorten recovery?

- a) Oral prednisolone
- b) IV methylprednisolone alone
- c) IVIG or plasmapheresis
- d) Azathioprine

Q19. A 28-year-old woman presents with painful red nodules over the shins, bilateral ankle arthritis, and dry cough. Chest radiograph shows bilateral hilar lymphadenopathy. The most likely diagnosis is:



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- a) Tuberculosis
- b) Sarcoidosis (Löfgren syndrome)
- c) GPA
- d) Histoplasmosis

Q20. A patient with pulmonary sarcoidosis develops recurrent renal stones. Which laboratory abnormality is most likely?

- a) Hyperkalemia
- b) Hypercalcemia
- c) Hypophosphatemia
- d) Hypernatremia

Q21. A 55-year-old man presents with progressive dyspnea. Thoracentesis reveals pleural fluid protein/serum protein ratio of 0.65 and pleural fluid LDH greater than two-thirds the upper limit of normal serum LDH. This pleural effusion is best classified as:

- a) Transudative
- b) Exudative
- c) Chylous
- d) Pseudochylous

Q22. A 67-year-old smoker with chronic cough and dyspnea undergoes spirometry. Post-bronchodilator FEV1/FVC ratio is 0.58. The most likely diagnosis is:

- a) Asthma
- b) COPD
- c) Bronchiectasis
- d) ILD

Q23. A 22-year-old woman experiences wheezing and chest tightness twice weekly. Lung function is normal between episodes. She has no nocturnal symptoms. According to current GINA



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recommendations, the preferred reliever therapy is:

- a) Salbutamol alone
- b) As-needed low-dose ICS-formoterol
- c) Oral prednisolone
- d) Montelukast alone

Q24. A 35-year-old man with asthma has FeNO of 65 ppb. Which of the following is most likely?

- a) Neutrophilic inflammation
- b) Type 2 eosinophilic airway inflammation
- c) COPD overlap syndrome
- d) Pulmonary fibrosis

Q25. A 68-year-old hypertensive man develops sudden onset aphasia and right hemiplegia while watching television. He reaches the emergency department within 90 minutes. CT brain shows no hemorrhage. The next best step is:

- a) Aspirin immediately
- b) IV thrombolysis after eligibility assessment
- c) Warfarin
- d) Mannitol

Q26. A 58-year-old man presents with dysphagia, hoarseness, vertigo, ipsilateral facial pain loss, contralateral body pain loss, and Horner syndrome. Which vessel is most likely occluded?

- a) MCA
- b) ACA
- c) PICA
- d) Lenticulostriate artery

Q27. A 5-year-old boy has frequent falls and difficulty climbing stairs. Examination reveals calf



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hypertrophy and a positive Gower sign. The most likely diagnosis is:

- a) Becker muscular dystrophy
- b) Duchenne muscular dystrophy
- c) SMA type III
- d) Myotonic dystrophy

Q28. A 22-year-old athlete presents with exertional syncope. Examination reveals an ejection systolic murmur that becomes louder during the Valsalva maneuver. The most likely diagnosis is:

- a) Aortic stenosis
- b) Mitral regurgitation
- c) HOCM
- d) ASD

Q29. A 75-year-old man presents with exertional angina, syncope, and dyspnea. Examination reveals a harsh ejection systolic murmur radiating to the carotids. The most likely diagnosis is:

- a) Aortic regurgitation
- b) Aortic stenosis
- c) Mitral regurgitation
- d) Pulmonary stenosis

Q30. A patient with severe aortic stenosis has a slow-rising, low-volume carotid pulse. This pulse is known as:

- a) Pulsus paradoxus
- b) Pulsus bisferiens
- c) Pulsus parvus et tardus
- d) Water hammer pulse

Q31. A 52-year-old woman has a pansystolic murmur best heard at the apex and radiating to the



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axilla. The most likely diagnosis is:

- a) Mitral stenosis
- b) Mitral regurgitation
- c) Aortic stenosis
- d) Tricuspid stenosis

Q32. A patient has a holosystolic murmur at the lower left sternal border that becomes louder during inspiration. This finding is known as:

- a) Austin Flint sign
- b) Graham Steell murmur
- c) Carvallo sign
- d) Gallavardin phenomenon

Q33. A 70-year-old man with complete heart block is noted to have intermittent giant pulsations in the neck veins. These are most likely:

- a) Giant V waves
- b) Cannon A waves
- c) Absent A waves
- d) Kussmaul sign

Q34. A 65-year-old man develops vomiting and diarrhea for 3 days. Laboratory tests show creatinine 2.3 mg/dL. Urine sodium is 10 mEq/L and FeNa is 0.5%. The most likely diagnosis is:

- a) Acute tubular necrosis
- b) Prerenal AKI
- c) Acute interstitial nephritis
- d) RPGN

Q35. A 58-year-old man with diabetes has eGFR 45 mL/min/1.73m² and urine albumin-creatinine



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ratio of 500 mg/g. Which drug has the strongest evidence for slowing progression of his kidney disease?

- a) Digoxin
- b) Ramipril
- c) Verapamil
- d) Hydrochlorothiazide

Q36. A 48-year-old man with CKD stage 3 is found to have normal serum calcium and phosphate levels. However, a biochemical abnormality is already present and contributes to CKD-mineral bone disease. Which of the following occurs earliest?

- a) Hyperphosphatemia
- b) Elevated PTH
- c) Increased FGF-23 levels
- d) Hypocalcemia

Q37. A 62-year-old diabetic man with CKD presents with fatigue and Hb of 8.5 g/dL. Iron stores are adequate. Peripheral smear shows normocytic normochromic anemia. The most likely mechanism is:

- a) Hemolysis
- b) Folate deficiency
- c) Reduced erythropoietin production
- d) Bone marrow infiltration

Q38. A 55-year-old man with CKD stage 5 is admitted with severe muscle weakness. ECG shows tall peaked T waves and widening of QRS complexes. Serum potassium is 7.2 mEq/L. The immediate stabilizing treatment is:

- a) Insulin-dextrose infusion
- b) Hemodialysis
- c) IV calcium gluconate



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d) Sodium polystyrene sulfonate

Q39. A patient with CKD has eGFR persistently below 15 mL/min/1.73 m² for the past 4 months. According to KDIGO classification, this corresponds to:

- a) G3a
- b) G3b
- c) G4
- d) G5

Q40. A 32-year-old woman with a history of rheumatic fever presents with exertional dyspnea and hemoptysis. Auscultation reveals a loud S1 followed by an opening snap and a low-pitched mid-diastolic murmur at the apex. The most likely diagnosis is:

- a) Mitral regurgitation
- b) Aortic stenosis
- c) Mitral stenosis
- d) Tricuspid stenosis

Q41. A 45-year-old woman with severe mitral stenosis develops sudden onset left-sided weakness. ECG shows atrial fibrillation. The most likely mechanism is:

- a) Septic embolism
- b) Cholesterol embolism
- c) Left atrial thromboembolism
- d) Vasculitis

Q42. A 40-year-old man presents with recurrent epigastric pain that improves after meals and recurs 2–3 hours later. Upper GI endoscopy reveals an ulcer in the first part of the duodenum. The most likely underlying etiology is:

- a) CMV infection
- b) H. pylori infection



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- c) Crohn disease
- d) Zollinger-Ellison syndrome

- Q43. A 25-year-old woman presents with chronic diarrhea, weight loss, and iron deficiency anemia. Endoscopic biopsy shows villous atrophy. Which serological test is most sensitive for diagnosis?
- a) ANA
 - b) Anti-dsDNA
 - c) IgA anti-tTG antibody
 - d) p-ANCA
- Q44. A 22-year-old woman presents with photosensitive rash, oral ulcers, arthritis, and proteinuria. ANA is positive. Which antibody is most specific for the diagnosis?
- a) Anti-Ro
 - b) Anti-La
 - c) Anti-dsDNA
 - d) Rheumatoid factor
- Q45. A 40-year-old woman presents with pain and swelling involving MCP and PIP joints of both hands. Morning stiffness lasts 2 hours. The most likely diagnosis is:
- a) Osteoarthritis
 - b) Rheumatoid arthritis
 - c) Gout
 - d) Psoriatic arthritis
- Q46. A patient suspected to have rheumatoid arthritis is tested for antibodies. Which antibody has the highest specificity?
- a) ANA
 - b) Rheumatoid factor



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- c) Anti-CCP antibody
- d) Anti-dsDNA

Q47. A newly diagnosed patient with rheumatoid arthritis has active synovitis involving multiple joints. Which is the preferred first-line DMARD?

- a) Methotrexate
- b) Colchicine
- c) Allopurinol
- d) Hydroxychloroquine alone

Q48. A 65-year-old man with HFrEF (EF 28%) is receiving guideline-directed therapy. Which of the following drugs has been shown to reduce mortality?

- a) Digoxin
- b) Sacubitril-valsartan
- c) Diltiazem
- d) Verapamil

Q49. A patient receiving unfractionated heparin develops life-threatening gastrointestinal bleeding. Which antidote should be administered?

- a) Vitamin K
- b) Protamine sulfate
- c) Idarucizumab
- d) PCC

Q50. A 55-year-old man sedated in ICU develops green-colored urine. He is hemodynamically stable and renal function is normal. Which drug is the most likely cause?

- a) Rifampicin
- b) Propofol



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- c) Digoxin
- d) Amiodarone

Q51. A 48-year-old woman has severe eosinophilic asthma despite high-dose ICS/LABA therapy. Blood eosinophil count is markedly elevated. Which biologic is most appropriate?

- a) Rituximab
- b) Infliximab
- c) Mepolizumab
- d) Abatacept

Q52. A 58-year-old diabetic man with CKD and albuminuria is started on a medication that lowers risk of CKD progression and cardiovascular events even in many non-diabetic CKD patients. Which drug is it?

- a) Acarbose
- b) Sitagliptin
- c) Dapagliflozin
- d) Glibenclamide

Q53. A 74-year-old man taking dabigatran presents with intracranial hemorrhage. Which specific reversal agent should be used?

- a) Protamine sulfate
- b) Vitamin K
- c) Idarucizumab
- d) Andexanet alfa

Q54. A patient on anti-tubercular therapy develops progressive loss of visual acuity and red-green color discrimination. Which drug is responsible?

- a) Rifampicin
- b) Isoniazid



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- c) Ethambutol
- d) Pyrazinamide

Q55. A pregnant woman with myasthenia gravis develops eclampsia. Administration of which drug may worsen neuromuscular weakness?

- a) Labetalol
- b) Magnesium sulfate
- c) Hydralazine
- d) Nifedipine

Q56. A 60-year-old man with septic shock remains hypotensive despite adequate fluid resuscitation. Which vasopressor is recommended as first-line therapy?

- a) Dopamine
- b) Phenylephrine
- c) Norepinephrine
- d) Ephedrine

Q57. A 68-year-old man with severe COPD has chronic resting hypoxemia (PaO_2 52 mmHg). Which intervention has been shown to improve survival?

- a) Tiotropium
- b) Salmeterol
- c) Long-term oxygen therapy
- d) Roflumilast

Q58. A patient presents with thyroid storm characterized by fever, tachyarrhythmia, delirium, and heart failure. Which antithyroid drug is preferred initially because it also inhibits peripheral T4-to-T3 conversion?

- a) Carbimazole
- b) Levothyroxine



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- c) PTU
- d) Radioiodine

Q59. A 35-year-old woman develops fever, arthralgia, ANA positivity, and anti-histone antibodies while receiving treatment for resistant hypertension. Which drug is the most likely cause?

- a) Losartan
- b) Hydralazine
- c) Amlodipine
- d) Metoprolol

Q60. A patient with severe pulmonary arterial hypertension is started on a drug shown to improve survival. Which drug has the strongest survival benefit evidence?

- a) Sildenafil
- b) Bosentan
- c) Epoprostenol
- d) Digoxin



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ANSWERS

1	b
2	b
3	b
4	b
5	b
6	c
7	b
8	b
9	b
10	c
11	b
12	b

13	b
14	a
15	b
16	b
17	c
18	c
19	b
20	b
21	b
22	b
23	b
24	b

25	b
26	c
27	b
28	c
29	b
30	c
31	b
32	c
33	b
34	b
35	b
36	c

37	c
38	c
39	d
40	c
41	c
42	b
43	c
44	c
45	b
46	c
47	a
48	b

49	b
50	b
51	c
52	c
53	c
54	c
55	b
56	c
57	c
58	c
59	b
60	c